SYNTHETIC CLINICAL STUDY PROTOCOL

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A Phase III Study of RSP-220 Inhaler in Adults With Moderate Persistent Asthma

Protocol ID: RSP220-305 **Study ID:** STUDY-006

Protocol version	2.3
Sponsor	Northwind Respiratory (synthetic)
Phase	Phase 3
Condition	Moderate Persistent Asthma
Design	Randomized, double-blind, placebo-controlled, parallel-group
Target enrollment	210
Duration	16 weeks
Primary objective	Change from baseline in pre-bronchodilator FEV1 at Week 16

1. Study Synopsis

This synthetic protocol evaluates RSP-220 in participants with Moderate Persistent Asthma. The study uses a randomized, double-blind, placebo-controlled, parallel-group design and plans to enroll approximately 210 participants across multiple sites.

2. Objectives and Endpoints

Primary endpoint: Change from baseline in pre-bronchodilator FEV1 at Week 16. Secondary objectives include participant safety, adherence, feasibility of remote data collection, and descriptive changes in study-specific patient-reported measures.

3. Study Arms

RSP-220 200 mcg inhaled twice daily; Placebo inhaler twice daily.

4. Inclusion Criteria

- Age 18-70 years
- Physician-diagnosed asthma for at least 12 months
- Pre-bronchodilator FEV1 50%-85% of predicted at screening
- Able to use a connected peak-flow meter

5. Exclusion Criteria

- Current smoker or smoking history of 10 pack-years or more
- Asthma exacerbation requiring hospitalization within 3 months
- Pregnancy

6. Study Procedures

Participants complete screening, informed consent, baseline assessments, scheduled study visits, and remote or site-based assessments according to the Schedule of Activities. Patient-reported measures may be collected through a secure messaging interface or linked web form.

7. Study Measurements

- Asthma symptom score: Daily (Patient message)
- Study inhaler used: Twice daily (Patient message)
- Rescue inhaler puffs: Daily (Patient message)
- Peak expiratory flow: Twice daily (Connected device)
- Spirometry FEV1: Weeks 0,4,8,16 (Site visit)

8. Safety Monitoring

Study staff review predefined safety and completeness signals. Automated software may flag missing or unusual values for human review but does not independently make medical decisions.

9. Data Management

Each participant is assigned a study-specific participant ID. Identifiable contact information is stored separately from the analysis dataset. Source responses, parsed values, timestamps, document versions, and audit events are retained.

10. Statistical Overview

The primary analysis will summarize the primary endpoint according to the prespecified analysis population. Descriptive statistics will be used for adherence, missingness, participant-reported measures, and operational monitoring.